

CONSULTATION NOTE — NEPHROLOGY OPD

Patient	Suresh Babu	Date	20-Mar-2025
Age/Sex	67 Years / Male	Visit	Follow-up — CKD Stage 4 monitoring
Consultant	Dr. Priya Nair, DM Nephrology	BP	148/92 mmHg
Weight	71 kg (prev: 74 kg)	Pulse	78 bpm, regular

SUBJECTIVE

67-year-old male presents for scheduled nephrology review. Complaints of increased fatigue and reduced appetite over the past 4 weeks. Urine output has decreased slightly. Bilateral ankle swelling returned despite Furosemide compliance. No chest pain or breathlessness. Gout: no acute episodes since starting Febuxostat and Colchicine. Blood sugar readings at home acceptable (FBS 110–130 mg/dL on Insulin Glargine). Weight loss of 3 kg in 2 months — partly dietary, partly likely uraemic anorexia.

CRITICAL SAFETY ALERTS — ACTIVE

ALLERGY: Sulfonamides — rash and angioedema. AVOID Trimethoprim-Sulfamethoxazole and all sulfa drugs.

CONTRAINDICATION: NSAIDs — absolutely contraindicated in CKD Stage 4. AVOID Ibuprofen, Diclofenac, Naproxen.

CONTRAINDICATION: Contrast media — high risk of contrast nephropathy. Pre-hydrate and use iso-osmolar contrast only if imaging essential.

CAUTION: Metformin — already stopped (eGFR < 30). Do not restart.

TODAY'S INVESTIGATIONS — TREND ANALYSIS

Test	Oct 2024	Feb 2025	Mar 2025	Trend
Serum Creatinine (mg/dL)	3.1	3.4	3.8	↑↑ WORSENING
eGFR (mL/min)	32	28	24	↓↓ Now Stage 4 — severe
Blood Urea (mg/dL)	54	61	74	↑↑ WORSENING
Serum Potassium (mEq/L)	5.6	5.2	5.4	BORDERLINE HIGH
Serum Bicarbonate (mEq/L)	—	—	18	LOW — metabolic acidosis
Haemoglobin (g/dL)	9.1	10.8	10.4	Stable — on EPO
Uric Acid (mg/dL)	9.8	7.2	6.1	IMPROVING on Febuxostat
24hr Urine Protein (g/day)	—	2.1	2.4	WORSENING — proteinuria
BP (mmHg)	—	136/86	148/92	↑ — needs adjustment

CRITICAL: eGFR now 24 mL/min. Rapid decline trajectory. Dialysis initiation likely within 3–6 months. RRT team referral urgent.

ASSESSMENT

- CKD Stage 4 (eGFR 24) — rapidly progressive. Metabolic acidosis developing.
- Hypertension — BP not at target (148/92). Telmisartan dose at maximum.
- CKD-related Anaemia — stable on EPO. Haemoglobin acceptable.
- Metabolic Acidosis — bicarbonate 18 mEq/L. Needs alkali supplementation.

5. Proteinuria increasing — 2.1 → 2.4 g/day despite ARB. Concerning.
6. Hyperuricaemia with Gout — well-controlled on Febuxostat.
7. T2DM — acceptably controlled on Insulin Glargine.

PLAN

- Add Sodium Bicarbonate 500mg TDS — for metabolic acidosis correction
- Add Carvedilol 6.25mg BD — second antihypertensive for BP control (replaces Metoprolol)
- Continue Furosemide 40mg OD — increase to 80mg if oedema worsens
- Continue EPO 4000 IU SC twice weekly + Iron supplementation
- **RRT (Renal Replacement Therapy) planning initiated today**
 - AV fistula creation surgery referred to vascular surgery (prepare access for haemodialysis)
 - Peritoneal dialysis discussed as alternative — patient to consider
- Renal diet: Strict low potassium (< 2g/day), low phosphorus, low protein (0.6g/kg/day)
- Nephrology review in 4 weeks — urgent if worsening symptoms
- **AVOID: NSAIDs, Sulfonamides, Contrast agents, Metformin, Aminoglycosides**